# Anamnesis

## Child Clinical Record

Standardized clinical framework for recording developmental history, psychomotor milestones, daily routine, and psychosocial context.

| Patient's Name | Age | Gender |
| --- | --- | --- |
| Lucas Gabriel Silva | 8 years and 4 months | Male |

| Guardian | Assessment Date |
| --- | --- |
| Helena Silva (Mother) | 10/01/2026 |

| Responsible Psychologist | Professional License (CRP) | Clinic / Institution |
| --- | --- | --- |
| Dr. Mariana Costa | CRP 06/142589 | Teko Neuropsychology Center |

Teko standardized clinical history assessment document.

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Clinical structure organized by child developmental stages, routine, and psychosocial context.

03

# Reason for Consultation

**Patient's Name**
Lucas Gabriel Silva

**Age**
8 years and 4 months

**Gender**
Male

**Guardian**
Helena Silva (Mother)

**Assessment Date**
10/01/2026

## Main complaint and detailed problem history

The mother reports that the child shows intense difficulty concentrating on school tasks, constant motor restlessness in the classroom, and impulsivity when interrupting conversations.

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# Pregnancy & Prenatal Care

**Was the pregnancy planned/desired?**

Yes

**Did the mother receive prenatal care?**

Yes

**Mother's age during pregnancy**

29 years

**Father's age during pregnancy**

32 years

**Complications and occurrences during pregnancy**

Mild hypertension and gestational anxiety monitored with continuous prenatal medical care.

05

# Childbirth & Birth

**Type of delivery**

Vaginal

**Cried immediately at birth?**

Yes

**Required neonatal ICU admission?**

No

**Presented jaundice at birth?**

Yes

**Birth weight (kg)**

3.250 kg

**Birth height (cm)**

49 cm

06

# Psychomotor Development

Age in months for developmental milestones

**Head control / Sitting unassisted**

6 months

**First words spoken**

11 months

**Crawling / Walking**

13 months

**Toilet training (daytime/nighttime)**

24 months

**Presented motor tics, habits or stereotypies?**

No

07

# Sleep & Nutrition

**Early childhood feeding pattern**

Breastfeeding

**Current nutrition and eating habits**

Good and balanced

**Is sleep peaceful and restorative?**

No

**Average sleep hours per night**

8 hours

**Sleeps alone in their own bedroom?**

Yes

**Observed sleep disturbances and nocturnal restlessness**

Shows nocturnal restlessness, occasional nightmares, and bedtime resistance.

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# Sexuality & Curiosity

**Natural sexual curiosity consistent with age?**

Yes

**Genital manipulation (masturbation)?**

No

**Received parental guidance at home?**

Yes

**Observations and reports regarding sexual behavior**

Natural and healthy curiosity consistent with childhood developmental stage.

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# Medical History & Health

**Previous medical conditions, respiratory or chronic**

Mild childhood bronchial asthma.

**Suffered head trauma or severe falls?**

**No**

**Suffers from migraines or recurrent headaches?**

**Yes**

**Ever experienced seizures, high fever or fainting?**

**No**

**Medical details, surgical procedures or hospitalizations**

Occasional tension headaches associated with academic fatigue at the end of the afternoon.

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# Family & Social Environment

Parents' marital status and co-parenting dynamic

**Living together**

**Relationship details with relatives and siblings**

Excellent relationship with parents and siblings. Harmonious and warm daily family atmosphere.

**Social interaction with peers (classmates and friends)**

Sociable and communicative, makes friends easily, but shows impatience during turn-taking games.

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# Schooling & Learning

Shows enthusiasm and motivation to attend school?

Yes

**School difficulties observed in the classroom**

Difficulty sustaining focus during continuous reading, essay writing, and mental arithmetic.

**Academic history, classroom behavior, and learning pace**

Attended early childhood education with positive social adaptation. No history of grade repetition.

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# Previous Treatments & Medication

**History of psychotherapy, speech or psychopedagogy therapy**

Previous psychopedagogical support for 6 months.

**Currently uses continuous prescribed medication?**

**No**

**Medication name, dosage, and medical monitoring**

No continuous medication prescribed at this time.

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# Final Observations & Conduct

**Preliminary diagnostic impression and clinical synthesis**

Clinical and behavioral presentation consistent with attentional instability and motor restlessness.

**Intervention plan, referrals, and clinical guidelines**

Referral for comprehensive neuropsychological assessment using Teko gamified digital biomarkers battery.

# Conclusion

**Clinical Synthesis & Conduct**

This clinical anamnesis brings together essential information to understand the child's holistic development, maturational milestones, physical health, daily routine, family dynamics, and educational background.

This standardized document provides empirical foundation and ethical alignment to contextualize neuropsychological telemetry screening, inform pedagogical strategies, and guide individualized care throughout clinical monitoring.

**Authenticity & Professional Responsibility**

Clinical anamnesis recorded electronically under professional responsibility of **Dr. Mariana Costa (CRP 06/142589)** on 10/01/2026. All information recorded adheres to ethical regulations and professional confidentiality.

Understanding to care.